Exam Case 1 (513)

CANDIDATE'S INSTRUCTIONS
A patient approaches you with a refill medication prescribed to him. Kindly counsel him as you would in regular practice. Also, assist her with any concerns she has regarding her condition

Time Frame: You have 7 minutes to complete this station.
References: CPS

Patient Record (Profile) Information:

Name: Kerry Ross

Age: 52

Diagnosis: Hyperlipidemia, Hypertension

Allergies: Sulfa drugs

Profile of prescriptions filled at pharmacy – last 5 months record

Drugs	Date filled	Directions	Qty	Refills
*pms-Amlodipine 10 mg tab	28/08/00	1 tab qd	90	3
*Crestor 10 mg tab	15/07/00	1 tab qhs	90	3
*ASA 81 mg tab	25/06/00	1 tab qd	60	5
*pms-Amlodipine 5 mg tab	18/06/00	1 tab qd	90	2
*Crestor 5 mg tab	15/06/00	1 tab qhs	90	2
*pms-Amlodipine 5 mg tab	15/06/00	1 tab qd	30	0

SP instructions:

•SP looks anxious . His both ankles are swelled up, he walks very slowly and he is limping a little. He inquires first if he has a refill in his file for Amlodipine 5mg. As he has refills left, he wants a refill for Amlodipine 5 mg tablets. But currently he has started Amlodipine 10 mg. When the pharmacist starts asking question he says ‘’ I am not fine with 10 mg dose. It dosen’t suit me. I want to take my old dose, I was doing completely fine with that ”

•Opening statement “Could you check if I have refills left for Amlodipine 5 mg tablets”

•If candidate asks about the dose change, SP says ‘ my BP wasn’t controlled it was 160/90. That’s why the dr. has changed the dose’. The BP has now for the last 5 days been pretty ok 140/85.

•If candidate says why do you want to take the old dose , SP says “ my legs are swollen up in the last few days, I barely can walk. I have heard that Norvasc gives side effects like this. Is it true?”

•If candidate says as you are on 10 mg right now, I cannot fill 5 mg for you…..SP says ‘why can’t you fill that as I still have refills on that”. If candidate says that he/she cannot fill prescription for 5 mg and change the dose without contacting the doctor…..SP says “ my doctor is on vaccation for 2 weeks, I cannot continue 10 mg anymore”

•If Candidate asks about Crestor, SP says he is not bothered about that. SP has been on it for 2 years also asks if the pharmacist by any chance know about any leg- swelling side effects from Crestor as well, or any worsening interactions between Crestor and Amlodipine.

•If candidate says “Do you monitor your BP regularly, SP says, “ I try to, but doesn’t happen always as I need to travel always”. SP is a medical representative.

•If Candidate asks about lifestyle, SP says I try to monitor diet. No smoking or alcohol. But I don’t have time for exercise”.

•If candidate says to continue 10 mg till dr. comes back….SP says “ I am in a lot of pain. My legs are already very bad. No way I will take it for a single day. Can’t you help me any way”?

•If candidate says to go to Walk-in-Clinic…..SP says ‘It already Sunday evening, everything is closed by now”. Also ask candidate has any thing he can suggested to help now.

•SP also asks the candidate “Can you tell me the name of any medication without this effect or which can reduce this effect, so that I can discuss about it with my dr.?”

TIPS

SOLVED:

•Analyzing the problem and not dispensing Amlodipine 5 mg

•Asking the patient about his **BP right now**, if he knows. Asking the patient if his BP was in control for the last few days after starting Amlodipine 10 mg. SP should say yes to this question.

•Explaining the patient about the relation between leg swelling and Amlodipine and it might take 7 days after the dose has been reduced by doc or changed.

Providing necessary Non-pharm – **elevation of the legs**

using compression stockings, avoiding prolonged periods with no movements

and reducing sodium in the diet.

MS SOLVED:

SOLVED:

•Analyzing the problem and not dispensing Amlodipine 5 mg

•Explaining the patient about the relation between leg swelling and Amlodipine and it might take 7 days after the dose has been reduced by doc or changed.

Providing necessary Non-pharm – elevation of the legs

using compression stockings,avoiding prolonged periods with no movements

and reducing sodium in the diet.

Pharmacy PREP OSCE Check List Case # 1 (513)

Case Title. Amlodipine

Communication skills (checklist)	**Outstanding**	**Good**	Acceptable	**NOT Acceptable**
Opening Statement (respond to patient clue, good listener, did not interrupt patient, engaging). Non-verbal Skills (eye contact, gesture, posture), Identified patient?	**4**	**3**	**2**	1
Information gathering process. (Asked M-A-M-A) allergies, medicine history, medical conditions. Builds trust and confidence with patient, Specific lifestyle such as alcohol and smoking habits prn; and/or other considerations relevant to the purpose of the case.	**4**	**3**	**2**	1
Discuss Follow up/Monitoring relevant to case	**4**	**3**	**2**	1
Time to check references, and use of proper references	**4**	**3**	**2**	1
Overall time management	**4**			
Overall communications	**Outstanding (20)**	**Good (15-20)**	Acceptable (12-14)	**(<14) NOT Acceptable**
Problem Solving (checklist)	Fully solved (FS)	Marginally solved (MS)	Marginally unsolved (MU)	Unsolved (U)
key elements of the counseling task 1. Analyzing the problem and not dispensing Amlodipine 5 mg 2.Asking the patient about his BP right now, if he knows. Asking the patient if his BP was in control for the last few days after starting Amlodipine 10 mg. SP should say yes to this question. 3.Explaining the patient about the relation between leg swelling and Amlodipine and it might take 7 days after the dose has been reduced by doc or changed. 4.Providing necessary Non-pharm – elevation of the legs using compression stockings,avoiding prolonged periods with no movements and reducing sodium in the diet.	All key points covered	3 key points were covered from 4 point	2 key points were covered from 4 point	Improper Recommendation
Misinformation?	**O**	**Put patients in to risk?**		**O**

Overall Performance	**Superior(O+FS)**	**Good(G/A +FS)**	**Fair(A +MS/MU)**	**Need to improve(U+MU/U)**

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Pharmacy PREP OSCE Check List. Case.

Communication skills	
	Information gathering process. (Asked M-A-M-A) allergies, medicine history, medical conditions. Builds trust and confidence with patient, Specific lifestyle such as alcohol and smoking habits prn; and/or other considerations relevant to the purpose of the case.
	Discuss Follow up/Monitoring relevant to case
	Time to check references, and use of proper references and Overall time management

Overall Communications	Acceptable 4/4	Marginally Acceptable 3/4	Marginally unacceptable 2/4	Unacceptable 1-0/4

Problem Solving
key elements of the counseling task

Overall Problem Solving	Fully solved 4/4	Marginally solved 3/4	Marginally unsolved 2/4	Unsolved 1-0/4
Note.	Misinformation?		Put patients in to risk?	O